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NAME: TEST PATIENT DELTA **AGE/SEX: 52 / F**
REFD. BY: TEST REFERRING DOCTOR **DATE: 15/10/2026**
UHID: TEST-004 **ACCESSION: ACC-MRI-LS-002**

MRI LUMBOSACRAL SPINE

CLINICAL HISTORY:

Chronic low back pain with left leg radiculopathy

TECHNIQUE:

Multiplanar, multisequence MRI of the lumbosacral spine was performed without intravenous contrast.

MEASUREMENTS:

Measurement	Value
L1-L2 Canal AP	14.5 mm
L2-L3 Canal AP	13.8 mm
L3-L4 Canal AP	13.2 mm
L4-L5 Canal AP	12 mm
L5-S1 Canal AP	11.5 mm

FINDINGS:

Loss of normal lumbar lordosis. Multilevel disc desiccation is noted from L2-L3 through L5-S1. At L4-L5, there is a diffuse disc bulge causing mild thecal sac indentation without significant neural foraminal narrowing. At L5-S1, there is a posterior disc protrusion measuring approximately 4 mm in AP dimension, causing moderate left neural foraminal narrowing with contact of the left L5 nerve root. Mild facet arthropathy is present at L4-L5 and L5-S1. The thecal sac is otherwise normal. The conus medullaris terminates at L1 level and demonstrates normal signal intensity.

IMPRESSION:

1. Loss of lumbar lordosis.

2. Multilevel disc desiccation.

3. L4-L5 diffuse disc bulge with mild thecal sac indentation.

4. L5-S1 posterior disc protrusion with moderate left neural foraminal narrowing and left L5 nerve root contact.

5. Mild facet arthropathy at L4-L5 and L5-S1.

RECOMMENDATION:

Clinical correlation for left L5 radiculopathy. Consider targeted epidural steroid injection if conservative management fails.

Dr. Sugandha Priyadarshini
MD (Radiodiagnosis & Medical Imaging)

Radiological diagnosis is not always conclusive & often vary with clinical course of the disease or response to treatment. This report is not for medico-legal purpose.